

African American Parents' experience with early childhood mental health and child welfare services: Racism and seeking understanding

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ABSTRACT

Background: Impacts of child maltreatment and child welfare involvement on young children are significant and early childhood mental health services are important to reduce risk factors for them. However, racial disparities limit access to mental health services.

Objective: The purpose of this study is to understand the experience and perspective of birth parents accessing mental health services for their child welfare-involved toddlers and preschoolers aged 1.5 to 5 years. Specifically, this study seeks to examine racism and service disparities among African American parents. To our knowledge, there is no study examining birth parents' perspectives on mental health services for young children.

Participants & Setting: This study collected original qualitative data using semi-structured interviews. Nine African American participants shared their experiences from February 2018 to November 2019.

Methods: Qualitative data analysis used a pragmatic realist approach to understand the complex phenomenon of parents' experience accessing services. NVivo 12 was used for qualitative analysis to help condense themes for the study. Quantitative descriptive analysis using Stata 14.2 provided the sample description.

Results: Equity and understanding was a main theme with three sub-themes; racism, child welfare understanding the family, and mental health provider understanding the family. Participants experienced racism, creating a mistrust of mental health providers and child welfare workers.

Conclusion: Implications for policy and practice to improve equity and understanding of families in child welfare are discussed. Future research can build on this study by examining how racism impacts service use and how interventions can increase understanding of families using child welfare and mental health services.

1. Introduction

The purpose of this study is to describe the experience and perspective of birth parents accessing mental health services for their child welfare-involved toddlers and preschoolers aged 1.5 to 5 years. In 2018, children birth to five years old represented 46 % of the 677,529 child maltreatment victims in the United States (U.S. Department of Health and Human Services [U.S. DHHS], 2020). Seventy percent of all fatalities resulting from maltreatment and abuse occur among children younger than 3 years old (U.S. DHHS, 2020). Children under age 3 represent approximately 28 % of maltreatment victims and infants are the fastest growing category to enter foster care (Groves, 2009; U.S. DHHS, 2020). The first five years of life build the foundation of a lifetime of social and emotional well-being. Children exposed to maltreatment in early childhood often experience trauma and are at risk for mental health problems and developmental delays (Barth et al., 2007; Cooper et al., 2010; Painter & Scannapieco, 2013; Whitaker, 2015). More than half (55 %) of children under age 3 in child welfare have at least five risk factors associated with poor developmental outcomes such as speech delays and hyperarousal to negative emotions (Cooper et al., 2010). The

impacts of child maltreatment on young children are significant and long lasting, making early intervention vital to reduce the risk of mental illness and medical problems throughout the lifespan (Kaplow & Widom, 2007; National Scientific Council on the Developing Child, 2020). Mental health and developmental services can help to reduce the traumatic effects of maltreatment in early childhood. However, despite the high need for these services, few children in child welfare receive them (Horwitz et al., 2012; Stahmer et al., 2005).

Many young children never receive services to meet these needs: 41 % of toddlers and 68 % of preschoolers in child welfare have a developmental or behavioral need, but only 24 % receive any type of service to meet those needs (Horwitz et al., 2013; Stahmer et al., 2005). Further, a national study of child welfare-involved children found that 42 % of children scored at a clinical level (≥ 64), indicating a significant level of challenging behavior, but only 28 % of children had received mental health services (Hurlburt et al., 2004). Only 24 % of child welfare-involved toddlers with an identified social and emotional problem received any type of mental health service. This discrepancy between need and services received highlights the potential that children in child welfare with mental health and developmental needs may not be

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receiving appropriate services, or any services at all to meet these needs.

The impacts of racism create intergenerational trauma for African American parents and their children. Structural racism (housing discrimination, lack of access to mental health treatment) and cumulative racial trauma (parental incarceration, racially motivated violence) interact to create a pathway for intergenerational depression (Hankerson et al., 2022). Experiences of racial discrimination are associated with parenting stress and depression, and 57 % of African American mothers of preschool aged children reported at least one experience of racial discrimination (Condon et al., 2021). African American parents' experience with racism is linked to parent and child conflict, and to their children's depression (Galán et al., 2022). African Americans resistance to substandard treatment approaches are often pathologized by mental health providers, rather than recognized as appropriate reactions to intergenerational trauma and oppression (Bailey et al., 2017; Shim, 2021).

Structural racism and individual racism combine to drive mental health inequalities (Mensah et al., 2021). Black caregivers have reported cultural mistrust of mental health services because of previous experiences of racism and bias by White mental health providers (Murry et al., 2011; Planey et al., 2019). Ben et al., (2017) conducted a meta-analysis and found racism was associated with more negative patient experiences of services including lower levels of trust, satisfaction, and communication. Studies have found racial disparities exist in mental health service utilization by children in child welfare. Black children have 56 % reduced odds (Stahmer et al., 2005) to 69 % reduced odds (Horwitz et al., 2012) to receive mental health services compared to White children. A recent study shows racial disparities with African American children involved with child welfare have 60 % reduced odds of being referred by caseworkers to mental health services (Fong et al., 2018). Racial disparities are present in child welfare investigations of child maltreatment. Compared with White children, Black children are at a higher risk for a child welfare investigation and later stage confirmation of maltreatment (Edwards et al., 2021). The mental health needs of young African American children in child welfare are clear, but the path to accessing and successfully completing mental health services is cluttered with ongoing barriers.

This study examines birth parents' experiences with racism and equity when accessing early childhood mental health services for their child welfare-involved children. The findings of this study will provide a unique contribution to understanding African American parents' experiences and perspectives on early childhood mental health services for their children involved in the child welfare system.

1.1. Literature review

1.1.1. Gateway providers and mental health service utilization

Despite child welfare policies supporting in-home services to allow children to remain safely placed in their birth parents' home (U.S. DHHS, 2020), children staying in their homes who have a mental health need are less likely to receive mental health services than children in out-of-home placements (Horwitz et al., 2012; Hurlburt et al., 2004; Ringeisen et al., 2009). Children with a mental health need living with a birth parent have lower odds of receiving mental health services compared to children living in out-of-home care (Horwitz et al., 2012; Ringeisen et al., 2009).

Conceptual models (e.g., Stiffman et al., 2004) include caregivers and providers as gatekeepers to children's mental health services. However, previous studies omit caregivers and providers as gatekeepers to children's mental health services. Conceptual models such as the Gateway Provider Model (Stiffman et al., 2004) were used in qualitative studies to understand the role of child welfare caseworkers in early childhood mental health service utilization. Caseworkers were interviewed to understand the barriers to delivery of early childhood mental health services (Hoffman et al., 2016). Without parents' perspective in early childhood mental health research, a gap exists in understanding

the factors involved with birth parents' engaging in formal mental health services for their children and their experience with these services.

1.2. Race and mental health service utilization

Child welfare caseworkers, mental health providers and parents are the gatekeepers or brokers of services for dependent children. Child welfare caseworkers connect children and their caregivers to needed mental health and developmental services by contacting providers, navigating waitlists, and providing transportation (Fitzgerald et al., 2015). However, child welfare caseworkers may also serve as barriers in some cases (Ayon, 2009; Gudino et al., 2012; Hoffman et al., 2016; Kelly & Barnard, 2000; Leslie et al., 2005; Padgett, 2016). Ethnic bias among professionals working with families can also create mistrust between a family and a provider. This finding suggests ethnic bias also needs to be assessed in studies regarding service utilization.

The research on racial disparities in mental health service utilization for toddlers and preschoolers involved in the child welfare system is limited. Horwitz et al. (2012) and Hurlburt et al. (2004) found race was a significant predictor, with Black children having lower odds of receiving mental health services than White children. However, Burns et al. (2004) and Ringeisen (2009) did not find race to be a significant predictor of mental health service utilization. This discrepancy between findings suggests mediating factors, such as agency collaboration, may impact racial disparities for children accessing mental health services. Hurlburt et al. (2004) found child welfare agencies with greater integration with mental health agencies experienced fewer racial disparities among African American children. There remains insufficient understanding of the causal pathways including individual, social, and organizational factors responsible for racial disparities in mental health service utilization (Garcia et al., 2012).

1.3. Research question

Current research on early childhood mental health service utilization is limited, creating an opportunity to study service delivery and improve children's mental health outcomes. Empirical evidence suggests children placed with their birth parents are less likely to receive services (Burns et al., 2004; Horwitz et al., 2012; Ringeisen et al., 2009); however, no studies have asked parents how they experience mental health services for toddlers and preschoolers while working with the child welfare system. Qualitative literature examining mental health service utilization focused on toddlers and preschoolers in the child welfare system is extremely limited. Hoffman et al., (2016) interviewed child welfare caseworkers regarding facilitators and barriers to mental health services for toddlers and preschoolers on their caseload. However, there are no known qualitative or mixed-methods studies exploring the experiences of parents of toddlers and preschoolers accessing mental health services.

This study seeks to expand the research knowledge about the experiences and perspectives of parents of toddlers and preschoolers when accessing mental health services. This study provides evidence of the lived experience of African American parents with racism, and equity and understanding in the child welfare and early childhood mental health systems. The primary research question is: What are the overall experiences and perspectives of parents when accessing services for mental health problems for their child welfare-involved toddlers and preschoolers?

2. Method

This study focuses on participant experiences and perspectives using Qualitative methods including a semi-structured interview to provide participant voice to the research. Primary studies of parents' experiences and perspectives when accessing mental health services are needed to

advance the conceptual framework of health behavior models and to improve child welfare and mental health services for children and families (Babitsch et al., 2012).

2.1. Study design

Using a primarily qualitative research approach is effective in understanding phenomenon where research is limited, such as participants' experiences and perceptions of early childhood mental health service utilization (Creswell & Plano Clark, 2017). Semi-structured interviews collected in-depth data on participants' perceptions and experiences in accessing mental health services. Semi-structured interview questions focused on participants' experiences with the systems they interact with, such as child welfare and mental health, as well as participants' own perspectives of children's mental health during early childhood. The semi-structured interview questions included prompts for participants to describe how their caseworker or mental health provider understood their family's culture or race. Parent demographics were measured using the parent survey. This study is part of a larger mixed methods study that used additional quantitative measures including The Child Behavior Checklist and the Youth Services Survey-Families. All study procedures were reviewed and approved by the University of Maryland, Baltimore, Institutional Review Board and the State's Department of Human Services (DHS) Social Services Administration (SSA).

2.2. Recruitment and sample

This study recruited participants from one community mental health center located in an urban center and four local Department of Social Services (DSS) located in surrounding suburban counties. Participants were recruited from February 2018 to November 2019. Two different recruitment methods were used for each type of study site. Recruitment procedures at the community mental health center were developed to ensure participants' involvement in the study was confidential. Participants only self-referred to the researcher. Study flyers were distributed to program managers of outpatient services such as mental health services and parenting services. Additionally, flyers were posted in the agency welcome area, where clients of the agency could view the flyer. Potential participants interested in the study contacted the researcher using the phone number listed on the flyer. The researcher conducted eligibility screening of potential participants by phone and, if eligible, the researcher briefly described the procedures for the in-person interview.

Recruitment procedures at the local DSS agencies were developed with the DHS to follow required research protocols for recruiting participants receiving services from the DHS. At local DSS sites, staff identified potential participants, handing them a flyer developed for use at local DSS sites. Interested participants completed a consent to be contacted form and gave the form to DSS staff. DSS staff sent the consent to be contacted to the researcher. The researcher contacted potential participants, assessed eligibility, and identified a location to meet for the interview.

Eligibility criteria included birth parents who accessed mental health services in the past 6 months or were currently engaged in mental health services with their child 1.5 to 5 years old (18 months to 71 months) and involved with child welfare in the past 12 months. Children could be six years old at the time participants consented for the study if they were under six years old when the child began mental health treatment. Mental health service utilization and child welfare involvement was measured by parent self-report during the phone eligibility screening and during the in-person interview. Participants had been involved in the child welfare system in the past 12 months when accessing mental health services to ensure recent child welfare involvement. Exclusion criteria included parents of children who had not been referred to, or participated in, mental health services and parents not comfortable

communicating in English.

The current study used purposeful sampling to identify 10 birth parents as participants. Ten participants completed the parent survey (Table 1) with nine participants completing the semi structured interview. All nine participants in the study identified as African American

Table 1
Sample demographics.

Variable	N(%)	X(SD)
Gender		
Female		
Male	9 (90 %)	
	1 (10 %)	
Race/Ethnicity		
African American		
African American & Japanese	9 (90 %)	
	1 (10 %)	
Parent age (years)		33.6 (7.98)
20–26		
27–33	1 (10 %)	
34–40	5 (50 %)	
41–46	2 (20 %)	
47–53	1 (10 %)	
	1 (10 %)	
Child age (months)		4.73 (1.13)
36–42		
43–49	2 (20 %)	
50–56	1 (10 %)	
57–63	1 (10 %)	
64–70	1 (10 %)	
71–77	0 (0 %)	
78–84	2 (20 %)	
	1 (10 %)	
Parent living arrangement		
Own home/apartment		
Rent home/apartment	2 (20 %)	
Homeless	7 (70 %)	
	1 (10 %)	
Marital status		
Married		
Never married	3 (30 %)	
Partnered	5 (50 %)	
	2 (20 %)	
Household annual income		
Less than \$10,000		
\$10,001 to \$20,000	3 (30 %)	
\$20,001 to \$30,000	1 (10 %)	
\$30,001 to \$40,000	2 (20 %)	
\$40,001 to \$50,000	0 (0 %)	
\$60,001 to \$80,000	1 (10 %)	
More than \$80,000	1 (10 %)	
	2 (20 %)	
Employment status		
Employed Full-Time		
Not employed, looking for work	4 (40 %)	
Not employed, not looking for work	4 (40 %)	
Not able to work due to disability	1 (10 %)	
	1 (10 %)	
Highest level of education		
Some High School (no degree): 9–12 grade		
High School degree/GED	1 (10 %)	
Vocational/technical training program	2 (20 %)	
Some college, no degree	1 (10 %)	
Bachelor's degree	3 (30 %)	
Master's degree or higher	1 (10 %)	
	2 (20 %)	
Child removed from home by child welfare		
Yes		
No	7 (70 %)	
	3 (30 %)	
Number of months child in out of home care		
0–5 months		
6–12 months	5 (50 %)	
12–18 months	1 (10 %)	
Not Applicable	1 (10 %)	
	3 (30 %)	

for their primary racial group, with one participant identified as Japanese for their secondary racial group. Results from the parent survey suggested the average participant was African American, female, 33 years old, never married, not employed, and had some college experience.

Participants with more than one child 1.5 to 5 years old, involved with child welfare, and attending mental health services were asked to select one child whose experiences accessing and using mental health services allowed parents to provide in-depth responses during the semi-structured interviews. For the current study, mental health services were defined as any supportive service addressing the identified child's social, emotional, mental health or behavioral need that involved the child individually or the parent and child together. Child welfare involvement is defined as any type of contact by the local DSS agency to investigate or provide services related to a child maltreatment report or concern.

The interviews took place in a confidential space convenient to participants, such as a public library, a university, and a restaurant. Once participants consented to be in the study, semi-structured interviews were conducted to obtain qualitative and quantitative data. Participant consent was also obtained to have the semi-structured interviews audio recorded. The interview time ranged from 28 min to 72 min. Questions during the interview followed an open-ended format for each question along with follow-up prompts to explore participants' responses. Participant responses around equity and racism came at various parts of the semi-structured interview. Questions about mental health providers and child welfare workers respecting families, understanding where they are coming from, and understanding families culture were often starting points for discussions around equity and racism.

2.3. Data analysis

The qualitative analysis was conducted using NVivo12 (QSR International, 2020) and the descriptive quantitative analysis for the sample description used Stata 14.2 (Stata Corp, 2015). The research question is addressed through data from the semi-structured interviews. The semi-structured interviews consisted of one-time interviews that were audio recorded and transcribed. Data from the semi-structured interviews were analyzed using a pragmatic realist approach (Miles et al., 2014). The pragmatic realist approach identifies regularities and sequences that link together phenomenon, and from these patterns, constructs are created that underlie individual and social life (Miles et al., 2014). The approach described by Miles et al. (2014) uses simultaneous data collection and analysis to inform each other, allowing the researcher to remain open to varied explanations or understandings of the data.

The qualitative data analysis occurred in five distinct phases. Transcripts from the semi-structured interviews were imported into NVivo 12. A provisional "start list" of codes (Miles et al., 2014) associated with mental health service utilization was used based on prior theory, research, and the conceptual model for this study. During the first cycle of coding, the authors read the transcripts line by line and used open coding; choosing words, phrases and sentences that matched the starter codes, or seemed to connect or relate to categories. Second cycle coding groups patterns or summaries into a smaller number of categories, themes, or concepts (Saldana, 2015). The focusing exercises further condensed codes by analyzing the patterns and comparing the patterns and codes to the research questions. A visual data display was created along with further reading of participant quotes, leading to the development of codes into patterns of data. The focusing strategies helped develop a cohesive thread connecting participant experiences in equity and racism while navigating mental health and child welfare services.

Data saturation in qualitative research is "data adequacy" or collecting data until no new information is obtained (Morse, 1995; Saunders et al., 2017). We used this definition of saturation to determine when there were no new development of themes in the analysis to add to the parents' experience accessing mental health services. For example, after the 9th interview, the data showed no new themes emerging and

the existing data collected sufficiently answered the research question to understand parents' experiences. We began a process to determine if data saturation was reached, then consulted with peers not affiliated with the study to determine the adequacy and depth of the data to add a significant contribution to the literature (Morse, 2015). The decision of saturation involved analyzing the context of the study. The data collection phase of this study was determined to be completed because of the adequacy of data saturation (Morse, 1995; Saunders et al., 2017) and the contribution to conceptualizing early childhood mental health services (Thorne, 2020).

Eliminating researcher bias in a primarily qualitative study is impossible so the goal is to understand researcher bias and use it productively (Maxwell, 2013). Using reflexivity, such as memoing, during the research process identified how bias may affect my interaction with participants. For example, the main researcher was a White male with clinical social work experience interviewing mostly African American female participants. Participants' trust may have been affected by answering questions from a White researcher about their experience with racism. Strategies to reduce researcher bias and reactivity in qualitative methods research aim to test the validity of conclusions made by the researcher (Maxwell, 2013). Creswell (2007) suggests several strategies to address threats to the validity of research including data triangulation, member checking, debriefing, and thick description. Member checking with participants occurred at the end of each interview. Themes and concepts discussed during the interview were shared with participants. Participants were also asked about the interview questions and process including how this study could be improved. Decisions were made transparent during this study by documentation through memos and journaling (Padgett, 2017).

3. Results

Semi-structured interviews using the interview guide were conducted with nine participants. Participants answered questions about their experience with child welfare and mental health services. During the qualitative interviews themes emerged from the participants' experience allowing for deeper analysis. The qualitative coding process from the larger study led to the themes in this study and is presented in Fig. 1. The overall theme in the qualitative analysis for this study is equity and understanding along with the sub-themes of racism, child welfare worker understanding family, and mental health worker understanding family. In this results section, nicknames for participants were used to provide cohesion and confidentiality of the participants shared experience.

3.1. Equity and understanding

Equity and understanding were identified as an overarching theme. Participants often described their mistrust of the system in their journey to seek help for their child and family. Participants continued to work with child welfare and mental health services to preserve their family. Participants desire to keep their family together allowed participants to cooperate with child welfare workers, despite participants belief the child welfare investigation was based on a false claim of child abuse. However, some participants describe experiences when child welfare understood their family and culture. Within this context, participants discussed their experiences of equity including how the child welfare and mental health system understood or misunderstood their family's culture. Participants also discussed how the African American community perceived child welfare and mental health. The overall theme of Equity and Understanding was developed from the semi-structured interviews and captured participants experiences with the following themes: 1) Racism; 2) Child welfare understanding family; and 3) Mental health provider understanding family.

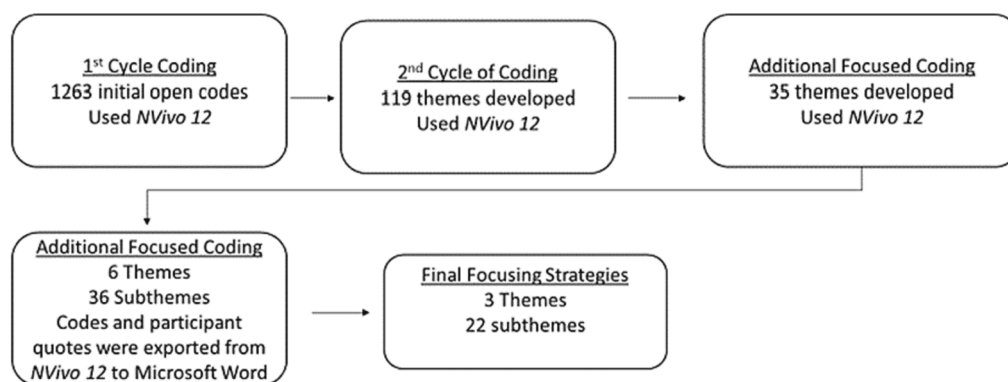


Fig. 1. Qualitative coding process.

3.2. Racism

Participants described their perceptions of racism and how they experienced it during their interactions with both child welfare and mental health providers. Racism in child welfare was described as systematic and based on the intersection of living in a low-income area of the metropolitan area, racist assumptions of the behavior of African American parents, and having low-income housing. Alexis stated, “Our perception in the Black community is that they’re aiming to destroy us. Because when you take away a mother’s child, that will destroy us very, very badly.” Sheila described racism as a factor in her involvement with child welfare:

“I feel like if I had not been a single Black mother in that building, I would not have been treated that way. Wouldn’t have gone that same way. It kind of was something that I feel like was easily swept under the rug because of honestly my race, and because of me living in West Baltimore and living in low income housing is very easy to get swept under the rug because you look like more of a number than a person. And as kind of how it felt is like, I was more of just a statistic than just a person with a family and real feelings. And it’s just the assumption that you must have been doing something wrong. And that really wasn’t the case. So that really did play a very big part in it.”

Participants encountered racism within the mental health system. Participants describe racism in mental health providers seeing the behavior of Black children as severe when compared to White children’s behavior. Alexis described a situation where a White child with extreme behavior was viewed by the mental health agency as having “extreme emotional problems” but not medicated and her child with less severe behavior needed medication. Alexis described her challenge of mental health providers bias towards certain diagnosis and medication for Black children:

“I feel like, not being biased or anything like that, but with the kids in our Black community they also just, she has ADHD, or he has ADHD. Let’s just get them on medication without really listening to what their needs are. Since I’ve been with [mental health agency] they’ve never mentioned nothing about medication, so I love [mental health agency].”

However not all participants identified racism within child welfare or mental health. It is important to consider the comfort level of participants talking about racism during the interview. When asked about experiencing racism from her child’s mental health provider, Shawanda stated, “Our culture is American, so it wasn’t any issue or anything.” Shawanda did not see racism as a problem for her because she identified with a mainstream American culture. It is possible during this interaction Shawanda was uncomfortable discussing her experiences of racism with me or she was responding to the question as related to broader cultural understanding.

3.3. Child welfare understanding family

Three participants described experiences when their interaction with child welfare was not equitable, and their needs were not understood. Participants knowing their rights during their interactions with child welfare was key to feeling empowered when interacting with the system. Jasmine reported she did not know her rights and “It turned up to a court situation, and the court’s not knowing how to work with people with disabilities, or the attorney not knowing how to work with people with disabilities. It can make it harder.” Participants did not trust the system to extend their rights to them, rather they had to demand their rights were respected. Alexis told her child welfare worker after her children were removed “You will never understand how that felt until you have kids.” Alexis described how her family’s need to have the kids together in foster care was not respected: “They separated my kids, they didn’t stay in the same house... For the system as well, I feel like they shouldn’t separate kids like that.” Removing a child from their parent can strain the relationship between the child welfare worker and the parent as Alexis stated: “I think that when they took my kids away that broke me mentally, physically and emotionally.”

Participants often acknowledged their limited autonomy to make decisions for their child in child welfare custody. However, when participants voiced their strong preferences for decisions about their child in foster care, they felt unheard when their preferences were not acted on by child welfare workers or foster parents. Sheila felt her child did not need a medical procedure because the medical records child welfare had access to were not a complete history. Sheila stated, “So I tried to protest and say he doesn’t need that, but my voice wasn’t heard. So that really was one of the other times where I was just like, I really wanted to be heard for that one.” Adi described his mixed reactions about his family’s culture being understood. Adi discussed participants understanding of the challenges faced by child welfare to make quick decisions about families, but at the same time the need to see each family and parent individually.

“Yeah and no because when you put ‘em in categories, it actually give you, like this research you’re doing. It actually give you a general idea, but at the same time, we have individuality, you know what I mean? So it’s good to approach things from a different standpoint, I’d say, when you’re dealing with different individuals, you know? You might have five people that use drugs, but all the five people probably don’t use drugs the same way, if you know what I mean.”

Feeling understood by child welfare workers was important to participants. Four participants described how child welfare understood family culture and participants experiences. Melanie was looking for work and described her interaction with the child welfare worker, “I think he understood where I was coming from, because they were trying to see if I was applying for jobs.” Amina felt child welfare understood her

family's need for protection and safety "They made sure we was safe from...her father so, they kind of also helped protected us too." Shawanda thought child welfare was investigating her after a false report. After child welfare worked with her family she felt the child welfare worker, "Understood that, Okay, this was clearly a false report and nothing is going on. So yeah. So she was really nice after that. Like once she saw the situation."

3.4. Mental health provider understanding family

Participants described how mental health providers understood their family's culture and experiences. Participants described mental health providers with similar backgrounds that made them feel understood and not judged. Natasha described her experience:

"He is from the inner city himself. Our backgrounds are somewhat similar culture wise. The thing that he does, he doesn't really assess or judge what I'm saying, he just gives me a different way of looking at the situation. Or helps me to be more considerate of the other side. That's something that's important."

During the time of the interviews, video of African Americans being killed and brutalized by police and violent racial attacks towards African Americans were circulated widely throughout social media and news outlets. In this context, Melanie felt it was important for her mental health provider to understand her African American child and having an African American mental health provider offered a deeper sense of understanding her child's experiences and challenges. Melanie stated:

"She's African American, and a number of her staff is African American. It's a diverse staff. That alone to me was like, "Okay." When she got down on the floor, and she was talking to him in a professional way, but just connecting with him, I said, "Okay, she gets that he's a little Black boy." I just think culturally she probably is more aware than if she wasn't African American, the importance of this session for him. I keep thinking about Black Lives Matter, and all those things."

Participants did not expect to be completely understood by mental health providers. When participants thought the mental health provider was trying to understand their family or experience, participants felt respected. Amina stated:

"They treated me with respect and stuff. Even if they probably didn't understand, ...they understood, if you get what I'm saying. So they like, they ain't gonna tell me they don't understand but they try to understand, you know. Like if I ask them a question, they give me their best response. Which, I think they understand cause you know the response they give me back. It was all respect."

4. Discussion

The purpose of this study was to understand the experiences and perspectives of parents accessing early childhood mental health services and those involved with child welfare. The qualitative approach used in this study allowed for a deeper understanding of participants' experiences accessing mental health and child welfare services.

4.1. Equity and understanding

All participants in this study identified as African American and described their experience with racism and equity in the mental health and child welfare systems. Black children are more likely than White children to experience traumatic stress but are less likely to be referred to and receive mental health services compared to White children (Fong et al., 2018; Garland et al., 2003; Gudino et al., 2012; Horwitz et al., 2012; Leslie et al. 2000; Morsy & Rothstein, 2019). Participants' children in this study received mental health services that many African

American children involved in child welfare never get (Fong et al., 2018; Horwitz et al., 2012; Stahmer et al., 2005).

For some African Americans, avoiding mental health services reduced exposure to additional structures reinforcing racial oppression (Alang, 2019; Planey et al., 2019). Racism creates mental health stressors for children (Condon et al., 2019; Trent et al., 2019) and additional barriers to accessing and remaining in mental health services (Alang, 2019; Planey et al., 2019). In 2016, Hoffman et al., found half of medical providers endorsed racist biological beliefs about Black patients such as tolerating a higher level of pain than White patients and having thicker skin than Whites. These beliefs led to less accurate treatment recommendations for Black patients compared to White patients. In 2020, African American physician Susan Moore was being treated at a hospital for COVID-19 symptoms. She complained she was not being prescribed appropriate medication and treated "Like a drug addict" by her attending physician who was White. She was discharged and died three weeks later (Maybank et al., 2020). Structural racism supports White racist beliefs about people of color in the mental health and child welfare systems (Stephens, 2022). In this study, one participant framed this example as child welfare trying to destroy the Black community by targeting herself and her neighbors for living in conditions created by structural racism such as substandard housing, lack of access to quality mental health services, and pathologizing adaptive resistance to child welfare services.

Some participants in this study perceived negative judgement about being single parents, parenting style, living environment and neighborhood. However, participants felt their concerns about their child's behavior were sometimes dismissed. One participant left a mental health agency because she felt they did not listen and were quick to put her African American child on medication instead of addressing the child's behavior issues. Racism experienced by participants from child welfare workers impacted their perceptions of mental health service referrals. For example, one participant felt her parenting behavior was judged negatively by child welfare because she is African American and living in low-income housing. When participants feel mistrust of child welfare stemming from racist interactions, referrals for mental health services from child welfare may be viewed with suspicion and mistrust.

African American parents report satisfaction and a quality therapeutic relationship when paired with an African American therapist (Cheng et al., 2021). Black parents reported a concern their child would not be treated equally by a White mental health provider (Planey et al., 2019). Participants in this study described feeling understood by African American mental health providers living in the metropolitan area. Participants felt African American mental health providers could understand the needs of their African American child, keep them safe, and acknowledge the racism they experienced. Racist and anti-racist interactions with professionals need to be considered for a complete understanding of families experiences with larger systems of care. The results of this study and prior research suggest equity is a factor in participants accessing mental health services. For example, facilitators of service use such as cultural competence of providers to help families build trust and barriers of service use such as racism both current and past perpetuate mistrust of providers.

4.2. Implications for research and practice

This study is unique because it provides the perspective of African American parents accessing early childhood mental health services for their children involved in child welfare. Participants' experiences with racism from the mental health and child welfare systems highlights the importance of anti-racist approaches to research and practice. Race can no longer be used as just another controlling variable but needs to be part of the overall study design. Including study participants in the research design and analyzing data by racial groups can help contextualize the impacts of racism in social work research studies. Future research can build on this study by examining barriers to understanding

the needs of families, and anti-racist interventions in child welfare and mental health agencies. Specifically, measuring how mechanisms in child welfare and early childhood mental health foster elements of structural and individual racism. Parents in this study who had positive relationships with their child welfare workers had positive experiences accessing mental health services and child welfare services. Relationships of child welfare case workers in medium and larger sample quantitative studies can be examined to test the association of relationship to case worker and access to mental health services and effective child welfare supports. Examining the role and relationships of gateway providers, and young children's access to early childhood mental health services is important.

Direct practice in early childhood mental health services is essential for infants, toddlers, and preschoolers to heal from trauma. Despite the negative impact of racism on African American youth mental health outcomes, interventions often ignore factors associated with racial trauma (Bernard et al., 2020; Metzger et al., 2020). Training in anti-racism is essential to both child welfare and mental health systems to reduce and eradicate racism as a barrier to treatment. Being "not a racist" will not eliminate racism, we must be anti-racist (Kendi, 2019). Racism impacts the mental health of African American families. Since 2015, an increase in the suicide of young African American children, specifically 5 to 11 years of age, is a concerning trend (Bridge et al., 2018; Congressional Black Caucus, 2019). All factors including the role of racism and mental health diagnoses needs to be considered in clinical practice.

Some participants in this study felt trust in having an African American therapist to understand the needs of African American children. Developing pathways for increasing the number of African American mental health and child welfare providers can help to improve workforce diversity and representation of the families they serve. This study also found participants were familiar and more trusting of services associated with familiar public institutions such as schools and libraries. Future studies are needed to identify the role schools and childhood education programs have in facilitating parent access to early childhood mental health services. With universal preschool becoming more common in the United States, research on early intervention can provide support for evidence-based interventions. Participants visited libraries for employment support, enrichment programs for young children, and access to social services. Library social workers need to continue engaging parents of children birth to five and support expansion for social work services in library settings (Soska & Navarro, 2020).

4.3. Strengths and limitations

Strengths. A major strength of this study is adding parents' perspectives to early childhood mental health research by exploring the parents' experiences accessing mental health services for children in birth to five involved with child welfare. To my knowledge, there have been no studies examining the parents' perspectives on mental health service use for toddlers and preschoolers despite early childhood intervention being a focus of public policy (e.g., early childhood special education in 1986, expansion of part C services in 1997; Kelly et al., 2000).

This study is important in bringing the voices of African American parents to early childhood mental health service use and child welfare services. African American parents have not been the focus of studies examining mental health service utilization despite research indicating Black children are overrepresented in child welfare (Dettlaff & Boyd, 2020) and are less likely to receive services than White children (Horwitz et al., 2012). During this study, the Black Lives Matter movement grew awareness of systemic racism, white supremacy, racial violence, and police brutality. Some participants in this study shared their experience of racism related to the child welfare system and the mental health system. This study provides valuable perspectives on the impact of racism on service utilization at a time when child welfare and mental health agencies are beginning to publicly evaluate their role in

perpetuating racism and white supremacy.

Limitations. As with all research, this study is not free from limitations. Small sample size from one state limits the generalizability of the results beyond the experience of the participants in the study. The sample does not include parents who were referred for mental health services and did not attend mental health services, nor children with an unidentified mental health need. Having this limitation omits the experiences of parents not choosing to attend services and thus the decisions not to seek services remain unknown. Another limitation to this study is having the parents' perspective without the perspectives of other stakeholders such as caseworkers, mental health providers, judges, and policy makers. Parents' perspectives of racism related to clinical factors such as mental health diagnosis and medication prescription for African American children may be missing alternative explanations from providers. Despite these limitations, this study fills the gap in research examining birth parents' experience with mental health services for young children involved in child welfare.

5. Conclusion

This study examined the perspectives and experiences of birth parents of children 18 months to 5 years old involved in early childhood mental health and child welfare. A qualitative methods approach was used to allow for a deeper understanding of parents' perspectives and experiences accessing early childhood mental health and child welfare services. Participants experienced racism when accessing mental health and child welfare services, creating mistrust of systems and impacted the relationships with child welfare workers and mental health providers.

Training in child welfare and mental health is needed for case-workers and mental health providers to build positive relationships with parents and develop an anti-racist organizational culture. Mental health providers working with African American families need to respect traditions, build strong relationships, and address racism and mistrust to allow for effective treatment. Child welfare workers and mental health providers have the unique position of reducing barriers related to racism and allow children to get help in addressing traumatic events and preventing additional maltreatment.

Declaration of competing interest

The authors declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

Data availability

The data that has been used is confidential.

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